DSM LAB · STUDY TOGETHER

# The Unfolding Patient: role-play cards

**Patients 13–24.**

One person plays the patient; the other is the social worker doing an intake. **Print single-sided**, so the answer page never sits on the back of the patient card.

## How to play

1. **Patient:** read the setting and your opening line aloud. Keep the rest of the card to yourself.
2. **Social worker:** interview. Ask about anything: onset and how long, sleep, substances and medications, medical history, safety, family, work or school, culture.
3. **Patient:** answer only what is asked, in your own words, using the "If asked about" table. Don't volunteer the items under "Only if asked".
4. **Social worker:** when ready (try 10 minutes), name a diagnosis and say what you'd do or check next.
5. Turn to the answer page together. Read the teaching point and the checkpoint question.

Each answer page follows its patient card. Keep it face down until the social worker commits to a diagnosis.

## Scoring, if you want it

3 points for the right diagnosis, plus 1 for naming the rule-out (substance, medication or medical cause) before deciding, plus 1 for a safe, assess-first next step. Swap roles any time.

## Playing it in the app

The same patients appear in DSM Lab → The Unfolding Patient, one clue at a time. If you play a card here first, the app version becomes a quick review.

These cases are original teaching material. Any resemblance to real people is coincidental.

# Patient 13: The license plate notebook

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Elias, 22, a former line cook from a farming town, at a telehealth intake from his rural health clinic. His father sits beside him.

> *"My dad wanted me to do this. The screen is fine, I guess, as long as it's not the news."*

**If asked about…**

| | |
|---|---|
| **When did this start?** | The voices, maybe six weeks ago. But I quit the restaurant way before that, about eight months ago. That's when I started noticing the plates. |
| **What do you hear?** | Two men arguing about me, like I'm a test they're grading. Every day, mostly at night. Nobody else hears them. |
| **What do you believe is going on?** | The news anchors send me messages, and the license plates are a code. Someone watches through my window, so I covered it. |
| **Sleep** | I sleep a lot, but it's broken up. I'm not tired or wired. Just flat. |
| **Mood** | Not sad, exactly. Nothing feels like much of anything. I've never had weeks of feeling really down or really up. |
| **Alcohol, drugs, medications** | I don't drink. Weed twice at sixteen, and I hated it. No pills. The ER test was clean. |
| **Medical history** | The ER did blood work and a scan of my head and said it was normal. No seizures, never hit my head hard. I'm healthy. |
| **Thoughts of suicide or harming others** | No. I don't want to die. The men argue; they don't tell me to do anything. I'm not going to hurt anybody. I just want them to leave me alone. |
| **Family history** | My dad's cousin lived in a group home. Nobody ever said why. |
| **Work and daily life** | I ran the grill on the night shift by myself. Now I don't do much. Dad reminds me to eat and shower. |
| **Culture and context** | Out here everybody knows everybody, and you handle your own problems. The nearest psychiatrist is three hours away, so this video call is it. |

**Only if asked (don’t volunteer)**

- Withdrawal, poor self-care and the license plate 'code' began about eight months ago
- Quit his job about eight months ago
- Father's cousin lived in a group home

# Patient 13: answer and debrief

ANSWER PAGE

DIAGNOSIS

## Schizophrenia

Delusions and voices with a clean screen, a normal workup and no mood episode point to a primary psychotic disorder, and six weeks of active symptoms first suggests schizophreniform disorder. Clue 5 flips it: withdrawal, poor self-care and odd beliefs about a 'code' began about eight months ago, and DSM counts this warning (prodromal) phase toward the 6 months of continuous signs. With a clear drop in work and self-care, the diagnosis is schizophrenia. Exam lesson: date the start of the illness, not just the start of the voices.

### What had to be ruled out

- Brief psychotic disorder: active symptoms have lasted more than 1 month
- Substance/medication-induced psychotic disorder: no current use, clean drug screen
- Psychotic disorder due to another medical condition: normal bloodwork and head scan
- Bipolar or depressive disorder with psychotic features: no manic or major depressive episode
- Schizoaffective disorder: no major mood episode at any point
- Schizophreniform disorder: total illness, counting the warning phase, is about 8 months

**Checkpoint: What would you ask or check NEXT?**

A. Record schizophreniform disorder (provisional) and begin psychoeducation with Elias and his father

B. Ask his father when Elias last seemed fully like himself and what changed in the months before the voices began **(best)**

C. Arrange an involuntary hospital admission because he believes he is being watched

With substances, medical causes and mood episodes addressed, the open question is the total length of the illness, and DSM counts early warning signs toward the 6 months. A names a diagnosis before the onset is known; a warning phase would change it. C jumps to the most restrictive option: a belief that he is being watched is not, by itself, danger to self or others or grave disability.

### The case in six clues (as the app reveals it)

1. I don't watch the news anymore. The anchors talk to me through the screen, and I'm tired of it.
2. Father: For about six weeks he's been hearing two men arguing about him when nobody's there. He covered his bedroom window with foil so they can't watch him.
3. I don't drink. I tried weed twice at sixteen and hated it. Father: The ER drug screen eight days ago was clean, and his blood tests and head scan were normal.
4. I'm not sad, and I'm not wired. I've never had weeks of feeling hopeless, and I've never felt on top of the world or gone days without sleep.
5. Father: It didn't start six weeks ago. About eight months ago he quit the restaurant, stopped seeing his friends and stopped showering. He'd sit for hours copying license plate numbers into a notebook, saying they were a code meant for him.
6. Father: There hasn't been one good week since then. He used to run the grill on his own; now I have to remind him to eat, and he barely talks.

### What a skilled clinician weighs after each clue

1. Schizophrenia, Brief psychotic disorder, Substance/medication-induced psychotic disorder, Bipolar I disorder
2. Schizophreniform disorder, Substance/medication-induced psychotic disorder, Schizophrenia, Bipolar I disorder
3. Schizophreniform disorder, Schizophrenia, Bipolar I disorder
4. Schizophreniform disorder, Schizophrenia
5. Schizophrenia, Schizophreniform disorder
6. Schizophrenia

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Schizophrenia

# Patient 14: The first-time mother

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Amara, 32, an elementary school teacher, at a postpartum follow-up visit at her obstetric clinic eight weeks after the birth of her first child. Her husband, Chidi, came with her.

> *"I'm fine now, truly. My husband wanted us to tell you everything, so here we are."*

**If asked about…**

| | |
|---|---|
| **When did it start and how long did it last?** | Five days after my son was born. It lasted about twelve days, then it lifted. I've been myself for over a month now. |
| **What happened?** | I was sure the nurses had switched our baby. A voice kept saying my real son was crying down the hall. I know now it wasn't real. It terrified me. |
| **Mood at the time** | I wasn't sad and I wasn't high. I wasn't talking fast or full of plans. I was scared. |
| **Sleep** | I slept whenever the baby slept. Now my mother and Chidi take turns at night, and I get decent sleep. |
| **Alcohol, drugs, medications** | Only prenatal vitamins before it started. I don't drink. In the hospital they gave me a medicine for the voice, and it was tapered off a week ago. |
| **Medical history** | A normal pregnancy and delivery. The ER checked my blood, thyroid and for infection. All normal, no fever. |
| **Thoughts of harming yourself or the baby** | No, never, then or now. I was afraid for him, not of him. Chidi or my mother was always with us, and I know to call right away if anything feels off. |
| **Past history** | Nothing like this ever. No high spells, no long sad spells. |
| **Family history** | None that I know of. I asked my mother, and she says no one. |
| **Functioning now** | I'm caring for the baby, cooking, calling my sister every day. I plan to go back to teaching when my leave ends. |
| **Culture and context** | My mother flew in for omugwo, our Igbo tradition where the grandmother cares for mother and baby after a birth. At first she thought it was spiritual and called our pastor. He told her to take me to the hospital. |
| **Future pregnancies** | We want another child someday. I'm scared this could happen again. |

**Only if asked (don’t volunteer)**

- No manic or depressive symptoms during the episode
- No personal or family history of mood or psychotic episodes
- Symptoms lasted about twelve days and fully resolved
- Worry about it happening in a future pregnancy

# Patient 14: answer and debrief



DIAGNOSIS

## Brief psychotic disorder

New delusions and a voice five days after delivery first suggest postpartum mania or depression with psychotic features, since bipolar disorder often first shows up this way. Clue 4 flips it: there was no manic or major depressive episode, only fear. Symptoms lasting about twelve days with full return to her usual self make it brief psychotic disorder with peripartum onset (onset during pregnancy or within 4 weeks after delivery). Exam lesson: psychosis after childbirth is a psychiatric emergency that needs same-day evaluation for the safety of parent and baby, and it raises the risk in later pregnancies, so plan ahead with her care team.

### What had to be ruled out

- Delirium: alert and oriented, no fever, normal labs
- Psychotic disorder due to another medical condition: normal thyroid, blood and infection tests
- Substance/medication-induced psychotic disorder: no alcohol, drugs or new medications before onset
- Bipolar I disorder: no manic episode, now or ever
- Major depressive disorder with psychotic features: no major depressive episode
- Schizophreniform disorder: symptoms lasted under 1 month with full recovery

**Checkpoint: What would you ask or check NEXT?**

A. Reassure her that these were the 'baby blues,' which are common and pass on their own

B. Refer her to a new-parent support group to rebuild her confidence

C. Ask whether she had a stretch of very high or very low mood around that time, and whether she or her relatives have ever had one **(best)**

With medical and substance causes cleared, the deciding question is whether the psychosis came with a mood episode; psychosis after childbirth is often the first sign of bipolar disorder. A is wrong: the blues are mild, short mood changes and never include psychosis. B may help later, but it skips the question that shapes the diagnosis, treatment and future risk.

### The case in six clues (as the app reveals it)

1. The first weeks after my son was born, I wasn't myself. Some of it scares me to think about.
2. Husband: Five days after the birth she started whispering that the nurses had switched our baby for another one. She said a voice kept telling her our real son was crying somewhere in the hospital.
3. Husband: We took her to the ER that night. Her blood tests, thyroid and infection checks were normal, she had no fever, and she knew where she was and the date. She takes only prenatal vitamins and doesn't drink.
4. I wasn't sad, and I wasn't racing. Husband: She slept whenever the baby let her, she wasn't talking fast or making big plans, and she didn't seem down. She was just frightened.
5. Husband: The voice and the switching idea lasted about twelve days in all. Then she held our son and said, 'He's ours.' She's been completely herself since, calling her sister every day and planning her return to teaching.
6. I've never had anything like this before, no high spells and no long low spells, and nobody in my family has. Husband: She took a medicine for the voice for several weeks. Her psychiatrist tapered it off a week ago, and she's still well.

### What a skilled clinician weighs after each clue

1. Major depressive disorder, Brief psychotic disorder, Bipolar I disorder, Delirium
2. Bipolar I disorder, Brief psychotic disorder, Major depressive disorder, Psychotic disorder due to another medical condition
3. Bipolar I disorder, Brief psychotic disorder, Major depressive disorder
4. Brief psychotic disorder, Schizophreniform disorder, Bipolar I disorder
5. Brief psychotic disorder, Bipolar I disorder
6. Brief psychotic disorder

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Brief Psychotic Disorder; ACOG Clinical Practice Guideline No. 4 (2023), Obstetrics & Gynecology 141(6), Screening and Diagnosis of Mental Health Conditions During Pregnancy and Postpartum (postpartum psychosis is an emergency); NIMH (2023), Perinatal Depression, NIH Publication No. 23-MH-8116, Baby blues: mild, short-lasting mood changes in the first 2 weeks after birth

# Patient 15: The folded towels



**Setting:** Diane, 57, manages a hospital pharmacy. She and her husband, Paul, came to a family service agency for couples counseling.

> *"I'm only here because Paul insisted. If he'd just tell the truth, we wouldn't need counseling."*

**If asked about…**

| | |
|---|---|
| **When did this start?** | About eight months ago. I noticed he'd changed. Thursday nights, the phone, the way she waves at him. |
| **What makes you sure?** | The seat in his car moved two notches. Leaves from her maple on his floor mat. The towels folded in thirds on Thursdays, when I fold them in half. Put it together. |
| **The investigator** | He found nothing, which tells you she got to him. I fired him. |
| **Could there be another explanation?** | No. I've looked at it from every angle. I know. |
| **Hearing or seeing things** | No, nothing like that. My mind is perfectly clear. |
| **Sleep and mood** | I sleep fine and eat fine. I'm not depressed. I'm angry. |
| **Alcohol, drugs, medications** | One glass of wine at book club. A blood pressure pill for ten years, same dose. Nothing new. |
| **Medical history** | A full physical two months ago, with blood work and a memory test. All normal. No head injuries, no seizures. |
| **Thoughts of harming Paul or the neighbor** | No. I'd never hurt anyone. I haven't said a word to her; I won't give her the satisfaction. There are no guns in our house. I just want him to admit it. |
| **Family history** | My aunt was 'odd,' lived alone and wrote angry letters to the paper. I don't know more than that. |
| **Work and daily life** | I manage twelve people, and my review this year was excellent. Book club every month. My life works, except for this. |
| **Culture and context** | We married in our Methodist church, and our faith matters to both of us. I never pictured anything but forever. Thirty-one years. That's why it hurts so much. |

**Only if asked (don’t volunteer)**

- Hired an investigator who found nothing; she believes he was paid off
- The belief began about eight months ago; she was trusting before
- Normal physical, blood work and memory test two months ago
- Aunt described as 'odd'

# Patient 15: answer and debrief



DIAGNOSIS

## Delusional disorder

An affair is possible, so the early clues fit a couple in conflict or a suspicious personality. Clue 3 flips it: she holds the belief against clear evidence and reads proof into trivial details. A fixed false belief for about eight months, new in a woman who was trusting for decades, with no prominent hallucinations or disorganization and her work life intact, fits delusional disorder, jealous type. Exam lesson: a first psychotic symptom after about 40 needs a medical and cognitive workup, and jealous delusions call for a private violence-risk check with each partner.

### What had to be ruled out

- No mental disorder: the belief is fixed despite clear contrary evidence
- Paranoid personality disorder: began about eight months ago after a lifetime of trust
- Schizophrenia: no hallucinations, disorganization or negative symptoms; functioning intact
- Major neurocognitive disorder: normal memory test; runs a department
- Psychotic disorder due to another medical condition: normal checkup and blood work
- Substance/medication-induced psychotic disorder: light alcohol, long-standing medication only
- Bipolar or depressive disorder with psychotic features: no mood episode

### Checkpoint: What would you ask or check NEXT?

A. Meet with each partner alone and ask about threats, violence, weapons and any thoughts of harming Paul or the neighbor **(best)**

B. Show Diane the investigator's findings again and help her weigh the evidence logically

C. Continue couples work on communication and rebuilding trust

Jealous delusions carry a real risk of violence toward the partner or the supposed rival, so a private safety check with each partner comes first. B argues with a fixed belief, which rarely changes it and can damage rapport. C treats a likely psychotic symptom as an ordinary trust problem and skips both the safety check and the diagnosis.

### The case in six clues (as the app reveals it)

1. Husband: We've been married thirty-one years, and for months now we fight every night. We need help.
2. He comes home late on Thursdays and keeps his phone face down. I know he's seeing the woman two doors down.
3. Husband: I'm at my brother's dialysis on Thursdays. Diane hired a private investigator, and he found nothing. Now she says the neighbor paid him off, and that the way the bath towels are folded proves I was with her.
4. Husband: She was never a suspicious person. For thirty years she trusted everyone. This started about eight months ago, and nothing I show her changes her mind.
5. I don't hear voices or see things, and my mind is clear. I'd never hurt anyone; I just want him to admit it. I still run a pharmacy with twelve people, my review this year was excellent, and I never miss my book club.
6. Husband: Her doctor did a full checkup with blood work and a memory test two months ago, all normal. She has a glass of wine at book club, takes the same blood pressure pill as always, and sleeps and eats fine. She isn't low or revved up, just angry with me.

### What a skilled clinician weighs after each clue

1. No mental disorder (normal or expected reaction), Delusional disorder, Paranoid personality disorder, Major depressive disorder
2. No mental disorder (normal or expected reaction), Delusional disorder, Paranoid personality disorder
3. Delusional disorder, Paranoid personality disorder, Schizophrenia, Major neurocognitive disorder
4. Delusional disorder, Schizophrenia, Major neurocognitive disorder, Psychotic disorder due to another medical condition
5. Delusional disorder, Psychotic disorder due to another medical condition, Major neurocognitive disorder
6. Delusional disorder

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Delusional Disorder

# Patient 16: The bar exam candidate

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Jordan, 26, a recent law school graduate studying for the bar exam, admitted to an inpatient psychiatric unit. The clues come from his first week there. His older sister and his unit nurse speak too.

> *"I feel almost normal again. I'm kind of embarrassed. I still don't totally get what happened to me."*

**If asked about…**

| | |
|---|---|
| **What happened?** | I was sure my classmates hacked my laptop and were livestreaming me. I heard them whispering in the vents and felt things crawling under my skin. I believed it completely. |
| **When did it start and stop?** | It came on over about two days, right before I called my sister. Here it faded, and by my fourth day it was gone. Now it seems ridiculous. |
| **Sleep** | Before I came in, four nights with almost no sleep. Here I slept twelve hours the first two nights. |
| **Alcohol, drugs, medications** | I bought stimulant pills online about three weeks ago to study longer. This past week I doubled up. I didn't think of them as drugs. A couple of beers on weekends. No other drugs, no prescriptions. |
| **Can you stop the pills?** | I want to. I told my sister to throw out what was left. I only started three weeks ago, so I've never tried to quit before. |
| **Medical history** | Healthy. They did blood tests and a heart tracing here. Normal, except my heart was racing when I came in. No seizures, no head injuries. |
| **Past mental health** | Nothing. I've never seen a therapist and never heard voices before this. |
| **Mood now** | Low and wiped out, honestly. I slept a ton the first days here. Mostly I'm ashamed. |
| **Thoughts of suicide or harming others** | No. I feel low and stupid, but I don't want to die. I'd never hurt my classmates; I was scared of them, not angry. |
| **Family history** | My uncle had a drinking problem. Nothing else that I know of. |
| **School and work** | I graduated near the top of my class. My job offer depends on passing the bar. That's why I was pushing so hard. |
| **Culture and context** | A lot of people in my study group talked about taking something to stay up. It felt normal. My sister is the only one I've told. |

**Only if asked (don’t volunteer)**

- Bought stimulant pills online about three weeks ago and doubled the dose this past week
- Almost no sleep for four nights before admission
- Low, tired mood since stopping

# Patient 16: answer and debrief



DIAGNOSIS

## Substance/medication-induced psychotic disorder

Sudden psychosis right before a high-stakes exam first suggests brief psychotic disorder with a marked stressor. Clue 5 flips it: a positive amphetamine screen and three weeks of escalating stimulant pills explain the paranoia, voices and crawling sensations. He believed the experiences were real, which goes beyond intoxication, and they cleared within days of stopping, so this is a stimulant-induced psychotic disorder. Exam lesson: ask about every substance, including diverted 'study drugs'; psychosis that lasts about a month or more after stopping suggests an independent disorder. Watch for the crash that follows, which can bring severe low mood and suicidal thoughts, and screen for a stimulant use disorder.

### What had to be ruled out

- Brief psychotic disorder: stimulant use explains the symptoms, which cleared with abstinence
- Stimulant intoxication: he believed the hallucinations and the hacking were real, and symptoms outlasted the drug
- Delirium: alert, oriented and attentive
- Psychotic disorder due to another medical condition: normal blood tests
- Schizophreniform disorder and schizophrenia: symptoms lasted days and fully cleared with abstinence
- Stimulant use disorder: not established by three weeks of use alone; screen for it, and if present it is recorded with the induced disorder, not instead of it

**Checkpoint: What would you ask or check NEXT?**

A. Tell his sister this is a stress reaction to the bar exam that should pass with rest

B. Ask about sleep and any substance use, including 'study drugs' and pills bought online, and request a urine drug screen **(best)**

C. Refer him to a long-term program for first-episode schizophrenia

Psychosis that came on over two days in someone with no history calls for checking substances and sleep loss first; crawling sensations in particular suggest a drug or medical cause. A assumes the cause and offers false reassurance. C names a long-term illness before the duration or the rule-outs are known.

### The case in six clues (as the app reveals it)

1. Sister: Jordan called me at 2 a.m. saying people were inside the walls of his apartment. I've never heard him so scared.
2. My classmates hacked my laptop and are livestreaming me. I hear them whispering through the vents, and I can feel tiny things crawling under my skin.
3. Sister: He's never had anything like this. He finished near the top of his class and has no mental health history. It all came on over about two days.
4. I haven't really slept in four nights. The bar exam is in nine days, and I have to keep studying. Nurse: He's alert, knows the date and where he is, follows our conversation, and his blood tests were normal.
5. Nurse: His urine screen came back positive for amphetamine. He told us he's been taking stimulant pills he bought online for about three weeks to study longer, and he doubled up this past week. Jordan: I didn't think study pills counted as drugs.
6. Nurse: He's had no pills for six days here. The whispering and crawling stopped by day four, and today he laughs about the livestream and says it couldn't have been real.

### What a skilled clinician weighs after each clue

1. Brief psychotic disorder, Schizophreniform disorder, Substance/medication-induced psychotic disorder, Delirium
2. Brief psychotic disorder, Substance/medication-induced psychotic disorder, Delirium, Schizophreniform disorder
3. Brief psychotic disorder, Substance/medication-induced psychotic disorder, Delirium
4. Brief psychotic disorder, Substance/medication-induced psychotic disorder
5. Substance/medication-induced psychotic disorder, Stimulant intoxication, Brief psychotic disorder
6. Substance/medication-induced psychotic disorder

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Substance/Medication-Induced Psychotic Disorder; SAMHSA TIP 33 (updated 2021), PEP21-02-01-004, Treatment for Stimulant Use Disorders

# Patient 17: The church organist

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Grace, 45, a piano teacher and church organist, at an integrated behavioral health visit in her primary care clinic. Her adult son, Isaac, came with her.

> *"Isaac made me come. The doctors say my water worries come from the seizures. I'm not so sure, but I'm drinking from the tap again."*

**If asked about…**

| | |
|---|---|
| **When did this start?** | The water worries started about four months ago. Isaac thinks the spells started around then too. The seizure medicine started five weeks ago. |
| **What do you believe is happening?** | That the neighbors put something in my water to make me sick. I saw their son near our meter. Lately I'm less sure. |
| **The spells** | Isaac says I stare and smack my lips. I don't remember that part. I remember a smell like burning rubber and a feeling that I've lived this exact moment before. Then I'm tired. |
| **Hearing or seeing things** | No voices, nothing like that. Just the smell before the spells. |
| **Sleep and mood** | I sleep fine. I'm not sad. I was frightened about the water, that's all. |
| **Alcohol, drugs, medications** | No alcohol, and I've never smoked anything. A cholesterol pill for six years, and now the seizure medicine. |
| **Medical history** | A bad car accident at nineteen. I hit my head and was in the hospital a few days, and I was fine after. The neurologist found a scar from that. |
| **Thoughts of suicide or harming others** | No, never. I wouldn't hurt the neighbors. I just stopped talking to them. |
| **Family history** | My mother had diabetes. Nothing like this in the family. |
| **Daily life and work** | I teach eleven students, play at church on Sundays and pay my own bills. The neurologist told me not to drive for now, so Isaac drives me. |
| **Culture and context** | Some people at church said to pray it away. My pastor said to pray and also see the doctor. I'm doing both. |

**Only if asked (don’t volunteer)**

- Staring spells with lip smacking that she does not remember
- A burning-rubber smell and a sense of having lived the moment before, just before spells
- Car accident with a head injury at nineteen

# Patient 17: answer and debrief



DIAGNOSIS

## Psychotic disorder due to another medical condition

A single persecutory belief with intact work and no hallucinations first looks like delusional disorder. Clue 4 flips it: brief staring spells with lip smacking, no memory of them and a burning smell beforehand are signs of temporal lobe seizures. A seizure focus on EEG, an old scar on MRI and a belief that fades once seizures are controlled show the psychosis is a physical effect of epilepsy. Exam lesson: new psychosis after about 40, smell or other non-auditory hallucinations, or spells of altered awareness call for a medical and neurological evaluation before any psychiatric diagnosis.

### What had to be ruled out

- Delusional disorder: the belief is explained by temporal lobe epilepsy and fades with seizure control
- Delirium: clear-headed and oriented between brief spells
- Major neurocognitive disorder: still teaches and manages her own finances
- Substance/medication-induced psychotic disorder: no alcohol or drugs; long-standing medication only
- Schizophrenia: one theme, no decline, and a clear medical cause
- Bipolar or depressive disorder with psychotic features: no mood episode

**Checkpoint: What would you ask or check NEXT?**

A. Diagnose delusional disorder and begin CBT for psychosis

B. Ask Isaac to have the water tested so his mother can be reassured

C. Refer her for a medical and neurological evaluation, since this is a first psychotic symptom at 45 **(best)**

A first delusion after about age 40 is a red flag for a medical or neurological cause, so evaluation comes before a psychiatric diagnosis. A may look right, but it skips the rule-out step. B is unlikely to shift a fixed belief and delays the workup.

### The case in six clues (as the app reveals it)

1. Son: For about four months Mom has said the neighbors are putting something in her water. She only drinks bottled water now.
2. I'm not confused about it. I still teach eleven piano students, keep my own books and play every Sunday. I just know what they're doing.
3. I don't drink or smoke, and I've taken the same cholesterol pill for years. I'm not depressed, and I sleep fine. Son: She's never had anything like this before.
4. Son: Twice in the past few months I've seen her stop mid-sentence, stare, smack her lips and pick at her sleeve for about a minute. Afterward she didn't remember it, and she said she'd smelled burning rubber right before.
5. Son: The neurologist's brain-wave test showed seizures starting in her left temporal lobe, and her MRI found an old scar from a car accident she had at nineteen.
6. Since I started the seizure medicine five weeks ago, the spells have stopped. Son: The water idea is fading too; yesterday she drank from the tap. Between spells she has always been clear-headed.

### What a skilled clinician weighs after each clue

1. Delusional disorder, Schizophrenia, Major neurocognitive disorder, Psychotic disorder due to another medical condition
2. Delusional disorder, Psychotic disorder due to another medical condition, Major neurocognitive disorder
3. Delusional disorder, Psychotic disorder due to another medical condition
4. Psychotic disorder due to another medical condition, Delusional disorder, Delirium
5. Psychotic disorder due to another medical condition, Delusional disorder
6. Psychotic disorder due to another medical condition

Sources: DSM-5-TR (APA, 2022), Schizophrenia Spectrum and Other Psychotic Disorders: Psychotic Disorder Due to Another Medical Condition

# Patient 18: The dairy farmer



**Setting:** Walter, 79, a retired dairy farmer and widower, in a skilled nursing facility for rehab after hip surgery. His daughter, Ruth, and the charge nurse speak too.

> *"Ruth worries too much. There's nothing to talk about. I've ruined everything."*

**If asked about…**

| | |
|---|---|
| **When did this start?** | About a week after I got here, everything went gray. Lately I understand the money is gone. |
| **Mood and interest** | Nothing matters. I don't follow the ballgames. Ruth brings the grandkids and I can't feel anything. |
| **Sleep and appetite** | I wake at three and lie there. Food sticks in my throat. I don't deserve it anyway. |
| **The money and the farm** | The bank took the farm. I can't pay for this place. Ruth shows me papers, but she's protecting me. |
| **The voice** | My father's voice at night, saying I shamed the family. He's been gone forty years. I only hear it at night, these last days. |
| **Thoughts of death or suicide** | Some nights I wish I wouldn't wake up. I'm not trying to die by not eating. No plan. I wouldn't do that to Ruth. My son-in-law keeps my old hunting rifles at his place. |
| **Alcohol and medications** | A beer at Christmas. Pain pills for a few days after the surgery, nothing since. A blood pressure pill and a baby aspirin. |
| **Medical history** | The hip and blood pressure. The nurse says my blood tests are good. My mind is fine; I did their memory test. |
| **Past episodes** | When my wife died I was like this, sure I'd lost everything. The doctor gave me pills, and in a few months I was fine. I've never had a time of being too high or wild. |
| **Family history** | My father was a hard man and drank. My sister had 'nerves' after her babies. |
| **Daily life before the fall** | I lived alone, drove to church and the feed store, and paid my own bills. |
| **Culture and context** | Farm people don't complain; you work. I've been at the same Lutheran church my whole life. My pastor visits on Tuesdays. |

**Only if asked (don’t volunteer)**

- Passive wish not to wake up; no plan or intent
- His rifles are kept at his son-in-law's home
- A similar episode with false beliefs at 66 that fully cleared with treatment
- Low mood began weeks before the beliefs and the voice

# Patient 18: answer and debrief



DIAGNOSIS

## Major depressive disorder

A voice and fixed beliefs in a 79-year-old after surgery first suggest delirium or dementia. Clue 3 flips it: weeks of low mood, lost interest, early waking and weight loss came first, and the beliefs of ruin, rot and shame match that mood. Normal labs, clear attention, a normal memory screen and a past episode in which mood and psychosis cleared together point to recurrent major depressive disorder with mood-congruent psychotic features. Exam lesson: men 75 and older have among the highest suicide rates, so ask directly; psychotic depression needs urgent psychiatric treatment, and refusing food can become a medical emergency.

### What had to be ruled out

- Delirium: alert, oriented and attentive; normal labs; no new medications
- Major neurocognitive disorder: normal memory screen; psychosis tied to the mood episode
- Substance/medication-induced depressive or psychotic disorder: pain pills only for a few days after surgery, weeks before onset
- Schizoaffective disorder: psychosis occurs only during depressive episodes
- Bipolar I disorder: no manic episode ever
- Adjustment disorder: full major depressive episode criteria are met

**Checkpoint: What would you ask or check NEXT?**

A. Ask Walter directly about thoughts of death or suicide, and whether not eating is a way to end his life **(best)**

B. Request a full dementia workup because of his age and the voice

C. Hold a family meeting about his wishes for end-of-life care and hospice

Severe depression with psychotic features in an older man who is refusing food is a high-risk picture, so suicide risk is assessed first. B delays the safety question; a focused medical check for delirium is still needed, but a full dementia workup is not the next step when the low mood clearly came before the psychosis. C treats food refusal as an informed choice when it may be a symptom of a treatable depression.

### The case in six clues (as the app reveals it)

1. Nurse: Since he came here for rehab after his hip surgery six weeks ago, he's eaten less and less and skips physical therapy.
2. Don't waste food on me. The bank took the farm, I can't pay for this room, and my insides are rotting. At night I hear my father's voice saying I've shamed the family.
3. Daughter: The low mood came first. About five weeks ago he lost interest in the ballgames, started waking at three, and dropped twelve pounds. The talk about the bank and the voice only started ten days ago.
4. Nurse: His blood count, sodium, thyroid, B12 and urine are normal. He only had pain pills for a few days after surgery, and nothing new has been started. He's alert, knows the date and where he is, and his memory screen was normal.
5. Sometimes I wish I wouldn't wake up. I'm not refusing food to die; I just don't deserve it. I have no plan, and I wouldn't do that to Ruth.
6. Daughter: At sixty-six, after Mom died, he had a spell like this and was sure he'd lost everything. With treatment his mood and those ideas cleared together, and he was himself for thirteen years. He has never had a high spell.

### What a skilled clinician weighs after each clue

1. Major depressive disorder, Delirium, Major neurocognitive disorder, Adjustment disorder
2. Delirium, Major neurocognitive disorder, Major depressive disorder, Psychotic disorder due to another medical condition
3. Major depressive disorder, Delirium, Major neurocognitive disorder
4. Major depressive disorder, Bipolar I disorder
5. Major depressive disorder, Bipolar I disorder
6. Major depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder

# Patient 19: The dental hygienist

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Renee, 38, a dental hygienist, at a telehealth visit through her health plan. She joins alone from her car on a lunch break.

> *"I almost canceled. It feels silly to talk about a breakup at my age."*

**If asked about…**

| | |
|---|---|
| **How long have you felt this way?** | If I'm honest, since about thirty. Most days are gray. Good stretches last a few weeks, never months. |
| **The breakup** | Daniel moved out three months ago. It hurt, but it's not like I was happy before. He used to say I was always on low battery. |
| **Mood and interest** | Low most days. I still enjoy pottery and cooking for my nephews. It just takes effort to get there. |
| **Sleep and appetite** | About seven hours, same as always. I eat normally. |
| **Energy and focus** | Tired all the time. I lose my train of thought with patients and recheck my charts. |
| **How you see yourself and the future** | I feel like I'm not worth much. I stopped going for promotions. What's the point? |
| **Alcohol, drugs, medications** | A glass of wine on Fridays. No drugs. No medications. |
| **Medical history** | My thyroid and blood count were normal two months ago. Healthy otherwise. |
| **High-energy periods** | Never. I've never needed less sleep or felt wired. |
| **Thoughts of suicide** | No. I've never wanted to die. I just want to feel the way other people seem to feel. |
| **Family history** | My dad was quiet and blue my whole childhood. Nobody called it anything. |
| **Culture and context** | In my family you 'offer it up' and don't complain. My mom thinks therapy is for people with real problems. |

**Only if asked (don’t volunteer)**

- Low mood most days since about age thirty
- Never felt well for more than a few weeks at a time
- Father with a similar long-term low mood

# Patient 19: answer and debrief



DIAGNOSIS

## Persistent depressive disorder

A recent breakup and fewer than the 5 symptoms MDD needs first suggest adjustment disorder. Clue 4 flips it: low mood most days for about eight years, never lifting for more than a few weeks, with low energy, poor focus, low self-worth and hopelessness. That meets persistent depressive disorder (2 years in adults, at least 2 symptoms, no symptom-free stretch over 2 months), with late onset because it began after 21. Exam lesson: 'I've been like this for years' points to PDD, and adjustment disorder is not used when another disorder explains the symptoms.

### What had to be ruled out

- Adjustment disorder: the low mood began years before the breakup
- Major depressive disorder: fewer than 5 symptoms; interest, sleep and appetite are intact
- Depressive disorder due to another medical condition: normal thyroid and blood count
- Substance/medication-induced depressive disorder: light alcohol, no medications
- Cyclothymic disorder and bipolar disorders: no periods of high energy or reduced need for sleep

**Checkpoint: What would you ask or check NEXT?**

A. Diagnose adjustment disorder and focus therapy on the breakup

B. Ask how long she has felt this way and whether the low mood started before the breakup **(best)**

C. Refer her for an antidepressant evaluation

Adjustment disorder is a residual diagnosis, so before tying symptoms to a stressor, find out when they really began. A assumes the breakup caused the symptoms. C is a treatment step taken before the diagnosis or its course is clear.

### The case in six clues (as the app reveals it)

1. I've been really down lately. My sister finally said I should talk to someone.
2. My fiance moved out about three months ago. Since then I drag myself to work and don't see the point of making plans.
3. I still love my Saturday pottery class and cooking for my nephews. I sleep and eat about the same, and I've never wanted to die. I'm just tired, I can't focus, and I feel like I'm not worth much.
4. Honestly, this isn't new. I've felt gray most days since I was about thirty. The longest I've felt good is a few weeks at a time, like the summer my nephew was born.
5. My doctor checked my thyroid and blood count two months ago, and both were normal. I have a glass of wine on Fridays and take no medications.
6. I've never had a stretch of feeling wired or needing less sleep. Even before he left, I'd stopped applying for promotions because I figured I'd fail.

### What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Persistent depressive disorder
2. Adjustment disorder, Major depressive disorder, Persistent depressive disorder
3. Adjustment disorder, Persistent depressive disorder, Depressive disorder due to another medical condition
4. Persistent depressive disorder, Depressive disorder due to another medical condition, Cyclothymic disorder
5. Persistent depressive disorder, Cyclothymic disorder
6. Persistent depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Persistent Depressive Disorder

# Patient 20: The garage project

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Owen, 31, an HVAC technician, at a behavioral health visit in his community health center. His husband, Jamal, came with him.

> *"Jamal thinks something's off with me. I figured I'm just a moody guy."*

**If asked about…**

| | |
|---|---|
| **When did this start?** | Around twenty-eight, so about three years ago. Before that I was pretty steady. |
| **The low spells** | About a week at a time. I sleep late, drag myself to work and feel like a loser. I can still laugh with friends, and it passes. |
| **The up spells** | Two days, three at most. I sleep five hours and feel great, I've got ten business ideas, and I tear into projects. Then I'm back to flat. |
| **What sets them off?** | Nothing I can point to. They just come. Not fights, not stress. |
| **How often?** | Jamal says I'm up or down more than half the time. The longest I've been just normal is a few weeks. |
| **Sleep** | Five hours when I'm up, ten when I'm low, eight in between. |
| **Alcohol, drugs, medications** | A couple of beers on weekends. No drugs. No medications. |
| **Medical history** | Physical two months ago. Thyroid and blood work normal. |
| **Thoughts of suicide** | No. Even on the low days I've never wanted to die. |
| **Family history** | My mom was up and down too. My grandfather was in the state hospital once. |
| **Work and money** | I've never missed work over it. When I'm up I order tools I don't need, maybe a couple hundred dollars. Jamal handles the budget now. |
| **Culture and context** | I'd rather fix a furnace than talk about feelings. Jamal grew up in a family that talks everything out, so he noticed the pattern first and made the appointment. |

**Only if asked (don’t volunteer)**

- Up spells of 2 to 3 days with little sleep and many ideas
- Symptoms more than half the time; never even for more than a few weeks
- Grandfather's psychiatric hospitalization

# Patient 20: answer and debrief



DIAGNOSIS

## Cyclothymic disorder

Three years of mild, recurring lows first suggest persistent depressive disorder. Clue 4 flips it: short spells of little sleep, big ideas and impulsive buying are periods of hypomanic symptoms. The highs last 2 or 3 days (hypomania needs 4) and the lows about a week (major depression needs 2), so neither reaches a full episode. With symptoms more than half the time for over 2 years and no even stretch longer than a few weeks, this is cyclothymic disorder. Exam lesson: count the days; a single full hypomanic or major depressive episode would change the diagnosis.

### What had to be ruled out

- Persistent depressive disorder: periods of hypomanic symptoms are present
- Bipolar II disorder: highs last 3 days at most and lows about a week; no full episodes
- Bipolar I disorder: no manic episode
- Substance/medication-induced bipolar disorder: no drugs or medications; light alcohol
- Bipolar and related disorder due to another medical condition: normal thyroid and blood work
- Borderline personality disorder: shifts last days, not hours, and are not set off by relationship stress

**Checkpoint: What would you ask or check NEXT?**

A. Recommend an antidepressant evaluation for his long-standing low mood

B. Begin CBT focused on his self-critical thoughts

C. Ask Owen and Jamal whether there are times his energy and mood run unusually high, and how long they last **(best)**

Before treating recurring low mood as depression, screen for highs: periods of hypomanic symptoms change the diagnosis and the treatment, and an antidepressant alone can push someone toward a high. A skips that screen. B may help later, but it starts treatment before the pattern is clear.

### The case in six clues (as the app reveals it)

1. I go through these flat stretches where I can't get going. Jamal says I'm moody.
2. When I'm low I sleep late, I'm tired, and I feel like a loser, but I still make it to work and I can laugh with friends. It's been like this, on and off, for about three years.
3. No drugs, a couple of beers on weekends, and no medications. My thyroid and blood work at my physical two months ago were normal.
4. Husband: Then out of nowhere he's up. He sleeps five hours, pitches business ideas at dinner, tears apart the garage and orders tools online. A couple of days later he's flat again.
5. The up spells last two days, three at most, and they've never gotten me in trouble. The low spells last about a week, never more than ten days, and I've never wanted to die.
6. Husband: In three years I can't remember him being just even for more than a few weeks. He's up or down more than half the time, but each spell is short.

### What a skilled clinician weighs after each clue

1. Major depressive disorder, Persistent depressive disorder, Bipolar II disorder, Cyclothymic disorder
2. Persistent depressive disorder, Cyclothymic disorder, Bipolar II disorder
3. Persistent depressive disorder, Cyclothymic disorder, Bipolar II disorder
4. Cyclothymic disorder, Bipolar II disorder, Borderline personality disorder
5. Cyclothymic disorder, Bipolar II disorder
6. Cyclothymic disorder

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Cyclothymic Disorder

# Patient 21: Fifty open tabs



**Setting:** Noa, 19, on a gap year and working at a climbing gym, at a walk-in crisis clinic. Her mother, Dana, came with her.

> *"I'm honestly amazing right now. I just need something for focus. Can you write me ADHD meds? No? Okay, who can?"*

**If asked about…**

| | |
|---|---|
| **When did this start?** | Like a week and a half ago? I woke up and everything clicked. I've never felt this alive. |
| **Sleep** | Two hours, maybe. I don't need more. I'm not tired at all. |
| **Focus and school history** | Now I can't stick with anything because I have too many ideas. In school I was fine. B's, organized, teachers liked me. |
| **Plans and spending** | Pro climber and music producer, both. I put $3,800 on Mom's card for gear and a studio deposit. It's an investment. |
| **The drive** | I drove to the coast to record the sunrise. The cop was dramatic. I was totally in control. |
| **Relationships** | I've been texting everyone. Two friends got weird and blocked me. Whatever, they're slow. |
| **Alcohol, drugs, medications** | A couple of drinks at a party a month ago. No weed, no pills, nothing prescribed. The urgent care test was clean. |
| **Medical history** | Healthy. Urgent care said my thyroid was normal. |
| **Past low or high spells** | In eleventh grade I was really down for a month or two and stayed in my room a lot. Nobody did anything about it. Never anything like now. |
| **Thoughts of suicide or harming others** | No way. I've never been happier. I'd never hurt anyone. |
| **Family history** | My mom's dad had 'episodes' and was in the hospital a few times. Mom doesn't like to talk about it. |
| **Culture and context** | Everyone on my feed has ADHD, and the videos describe me exactly. My mom thinks it's because I'm off my routine on a gap year. |

**Only if asked (don’t volunteer)**

- Sleeps about two hours a night and feels rested
- No attention problems in childhood
- A low period of a month or two in eleventh grade
- Grandfather with psychiatric hospitalizations

# Patient 21: answer and debrief



DIAGNOSIS

## Bipolar I disorder

Distractibility, restlessness and impulsive texting look like ADHD or borderline personality disorder at first. Clue 4 flips it: this is a sharp change ten days ago in a previously calm, organized young woman, with little need for sleep. ADHD needs symptoms before age 12, and borderline needs a long-standing pattern. Grandiose plans, heavy spending, pressured speech and dangerous driving for over a week with marked impairment make this a manic episode, so the diagnosis is bipolar I. Exam lesson: ask when it started and how the person sleeps; mania can be a safety emergency that needs urgent psychiatric care, and a stimulant would be the wrong treatment.

### What had to be ruled out

- ADHD: no symptoms in childhood; a sudden change ten days ago
- Borderline personality disorder: no long-standing pattern; a distinct episode with reduced need for sleep
- Substance/medication-induced bipolar disorder: negative drug screen; no medications
- Bipolar and related disorder due to another medical condition: normal thyroid
- Bipolar II disorder: marked impairment and danger make this mania, not hypomania

**Checkpoint: What would you ask or check NEXT?**

A. Ask when these changes began, how she has been sleeping, and whether she has ever been like this before **(best)**

B. Refer her to a prescriber for an ADHD stimulant trial

C. Enroll her in a DBT skills group for impulsivity and relationship conflict

ADHD starts in childhood and is steady; bipolar disorder comes in episodes, often with less need for sleep. Asking about onset and sleep separates them. B could worsen a manic episode if one is present. C targets a personality pattern before anyone knows whether this is new or long-standing.

### The case in six clues (as the app reveals it)

1. I can't focus on anything. My brain has fifty tabs open. I'm pretty sure I have ADHD, and I need medication.
2. Mother: She quit her job, she's lost her keys three times this week, and she can't sit through a meal. She's been texting people she barely knows at all hours, and two friendships have blown up.
3. I don't do drugs. I had a couple of drinks at a party a month ago. Mother: The urgent care checked her this morning; her drug screen was negative and her thyroid was normal.
4. Mother: This isn't how she's always been. She was a calm, organized kid, and no teacher ever raised a concern. This started about ten days ago, and she sleeps two hours a night without seeming tired.
5. I've never felt this good in my life. I figured out I'm meant to be a pro climber and a music producer at the same time. I put $3,800 on Mom's card for gear and a studio deposit. Mother: She talks so fast I can't get a word in.
6. Mother: Last night she drove to the coast at 3 a.m. to record the sunrise and got pulled over doing over a hundred. That's when I said we were getting help.

### What a skilled clinician weighs after each clue

1. ADHD, Bipolar I disorder, Stimulant intoxication, Generalized anxiety disorder
2. ADHD, Borderline personality disorder, Bipolar I disorder, Stimulant intoxication
3. ADHD, Borderline personality disorder, Bipolar I disorder
4. Bipolar I disorder, Bipolar II disorder
5. Bipolar I disorder, Bipolar II disorder
6. Bipolar I disorder

Sources: DSM-5-TR (APA, 2022), Bipolar and Related Disorders: Bipolar I Disorder

# Patient 22: The well-baby visit

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Megan, 33, a veterinary technician, at her daughter's well-baby visit in a pediatric clinic. The clinic's behavioral health social worker meets with her and her wife, Tasha.

> *"The pediatrician asked how I was doing and then asked me to talk to you. I'm fine. I'm just tired."*

**If asked about…**

| | |
|---|---|
| **When did this start?** | A few days after we got home. Everyone said baby blues. It just never stopped. It's been about six weeks, and worse lately. |
| **Mood and interest** | Low almost every day. I don't enjoy anything, not even holding her. That's the worst part. |
| **Sleep** | Tasha does the night feeds so I can sleep, and I just lie there. Maybe three or four hours. |
| **Appetite** | Food doesn't interest me. I skip meals unless Tasha puts a plate in front of me. |
| **How you see yourself as a parent** | I feel like a terrible mother. I can't decide anything, even which bottle to use. |
| **Thoughts of harming yourself or the baby** | Sometimes I think she'd be better off with a mother who isn't me. I would never hurt her. I don't have a plan to hurt myself, and I'd tell Tasha if that changed. |
| **Unusual experiences** | No, I don't hear or see anything strange. I know these thoughts come from feeling low. |
| **Alcohol, drugs, medications** | Just prenatal vitamins. I don't drink, especially while nursing. No other medications. |
| **Medical history** | A normal delivery. My doctor checked my thyroid and blood count eight days ago; both were normal. |
| **Past mood history** | I had a rough few months in college after my dad died, and I got through it. I've never had a time of feeling super energized or not needing sleep. |
| **Family history** | My mom says she 'wasn't right' for a year after I was born. |
| **Support and context** | Tasha is amazing. Some of my family still doesn't accept us, so we don't have much help. I feel like I have to prove we can do this. |

**Only if asked (don’t volunteer)**

- Thoughts that the baby would be better off with another mother; no intent or plan
- Sleeps only three or four hours even when given the chance
- A depressive period in college
- Mother's likely postpartum depression

# Patient 22: answer and debrief



DIAGNOSIS

## Major depressive disorder

Tears in the first days after birth are often the baby blues: mild mood swings that usually fade within about two weeks without impairment. Clue 3 flips it: her low mood never lifted and has worsened for six weeks. Six symptoms nearly every day, starting within 4 weeks of delivery, with a normal thyroid and no history of highs, make this major depressive disorder with peripartum onset. Exam lesson: blues are short and mild; low mood that lasts past two weeks or causes impairment needs assessment, including direct questions about harming self or baby.

### What had to be ruled out

- No mental disorder (baby blues): symptoms have lasted far longer than two weeks and cause impairment
- Adjustment disorder: full major depressive episode criteria are met
- Depressive disorder due to another medical condition: normal thyroid and blood count
- Substance/medication-induced depressive disorder: only vitamins, no alcohol
- Bipolar II disorder: no past high spells
- Psychotic features: no hallucinations or delusions; she sees her thoughts as part of the depression

**Checkpoint: What would you ask or check NEXT?**

A. Reassure them that the baby blues are common and recheck her mood at the next well-baby visit

B. Ask Megan about her symptoms, including any thoughts of harming herself or the baby, using a validated screen such as the EPDS **(best)**

C. Refer her to a new-parent support group

Low mood that has lasted six weeks and is getting worse is well past the window for the blues, so it needs a full assessment now, including safety for parent and baby. A delays care for a likely depression. C may help as an add-on, but it is not an assessment and skips the safety questions.

### The case in six clues (as the app reveals it)

1. I cry a lot. Everyone says that's normal with a new baby.
2. Wife: Megan gave birth to our daughter seven weeks ago. The crying started a few days after we came home from the hospital.
3. Wife: Everyone told us it would pass in a week or two, but it never did. She's been low nearly every day for six weeks, and it's getting worse.
4. I can't sleep even when the baby sleeps. I don't enjoy anything, even holding her. I have no appetite and skip meals, I can't make simple decisions, and I feel like a terrible mother.
5. Sometimes I think she'd be better off with a mother who isn't me. I would never hurt her, and I have no plan to hurt myself. I don't hear or see anything strange; I know these thoughts come from feeling so low.
6. Wife: Her doctor checked her thyroid and blood count eight days ago, and both were normal. She takes only vitamins, doesn't drink, and has never had a spell of high energy or racing thoughts.

### What a skilled clinician weighs after each clue

1. No mental disorder (normal or expected reaction), Major depressive disorder, Adjustment disorder
2. No mental disorder (normal or expected reaction), Major depressive disorder, Adjustment disorder
3. Major depressive disorder, Adjustment disorder, Depressive disorder due to another medical condition
4. Major depressive disorder, Depressive disorder due to another medical condition, Bipolar II disorder
5. Major depressive disorder, Depressive disorder due to another medical condition, Bipolar II disorder
6. Major depressive disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder (with peripartum onset); ACOG Clinical Practice Guideline No. 4 (2023), Obstetrics & Gynecology 141(6), Screening and Diagnosis of Mental Health Conditions During Pregnancy and Postpartum (validated screening such as the EPDS); NIMH (2023), Perinatal Depression, NIH Publication No. 23-MH-8116, Baby blues: mild, short-lasting mood changes in the first 2 weeks after birth

# Patient 23: The wrestling coach

PATIENT CARD · FOR THE PERSON PLAYING THE PATIENT

**Setting:** Yusuf, 56, a high school history teacher and wrestling coach, referred to the behavioral health social worker in his primary care clinic. He describes the past several months. His wife, Amina, came with him.

> *"Amina says I should tell you how it was. I'm better now, mostly, but those months were rough."*

**If asked about…**

| | |
|---|---|
| **When did it start and how long did it last?** | It crept in about four and a half months ago. It was bad for about three months, until the thyroid test. It's been lifting over the last few weeks. |
| **Symptoms at the worst** | No joy in coaching, ten hours of sleep and still tired, fifteen pounds heavier, and I felt useless in class. Every day. |
| **Physical changes** | I was cold all the time, my skin was dry, my voice was hoarse, I was constipated, and my face got puffy. I thought it was just getting older. |
| **Sleep now** | About eight hours, and I wake up rested most days. |
| **Alcohol, drugs, medications** | No alcohol, ever. No drugs. The same blood pressure pill for eight years, and now the thyroid pill. |
| **Medical history** | Blood pressure. The doctor said my thyroid was very low, an autoimmune type. She checks my levels every few weeks. |
| **Mood now** | Much better. I'm back at practice and I laugh again. Maybe not a hundred percent, but close. |
| **Past mood history** | Never depressed before, never had a high spell. I've always been an even guy. |
| **Thoughts of suicide** | At the worst I felt useless, but I never wanted to die or thought about ending my life. I have my kids, my team and my faith. |
| **Family history** | My mother had thyroid problems too. No mental illness that I know of. |
| **Work** | I kept teaching, but barely. A colleague covered practice for two months. |
| **Culture and context** | In my family a man doesn't complain about feelings. I told the doctor I was tired, not that I was sad. Amina is the one who said the word depressed. |

**Only if asked (don’t volunteer)**

- Feeling cold, dry skin, hoarse voice and constipation
- Mood lifted as thyroid levels improved
- Mother had thyroid disease

# Patient 23: answer and debrief



DIAGNOSIS

## Depressive disorder due to another medical condition

Five or more symptoms every day for three months fit a major depressive episode, so MDD leads. Clue 4 flips it: feeling cold, dry skin, a hoarse voice, puffiness and slowed thinking are classic signs of an underactive thyroid, and the blood test confirms it. A first depression that came with the thyroid problem and lifted as it was treated points to depressive disorder due to another medical condition (hypothyroidism). Exam lesson: refer for a medical evaluation before calling it MDD, especially with physical symptoms or a first episode in midlife; if the depression lasted after his thyroid levels were normal, an independent MDD would be considered.

### What had to be ruled out

- Major depressive disorder: the depression began with hypothyroidism and lifted as it was treated
- Substance/medication-induced depressive disorder: no alcohol or drugs; same medication for eight years
- Adjustment disorder: no stressor
- Bipolar II disorder: no past high spells
- Persistent depressive disorder: about three months, not 2 years

**Checkpoint: What would you ask or check NEXT?**

A. Refer him for an antidepressant evaluation

B. Start behavioral activation focused on getting back to coaching

C. Ask about physical changes and when he last had a medical checkup with blood tests **(best)**

A first depression in midlife with heavy fatigue, oversleeping and weight gain calls for a medical check first; hypothyroidism, anemia and sleep apnea are common mimics. A and B treat a primary depression before a medical cause has been ruled out.

### The case in six clues (as the app reveals it)

1. For about three months I felt flat and slow, like I was walking through mud.
2. I stopped enjoying coaching, I slept ten hours and woke up tired, I gained fifteen pounds, and I felt useless in my classroom. It was like that every day.
3. No alcohol, no drugs, and I take the same blood pressure pill I've taken for eight years. Nothing bad happened; work and home were fine.
4. Wife: He was cold all the time, even in a hot gym. His skin got dry, his voice went hoarse, his face looked puffy, and his thinking slowed down.
5. The blood test showed my thyroid was very underactive, an autoimmune kind. My doctor started me on thyroid hormone six weeks ago.
6. Wife: As his levels came back toward normal, the old Yusuf came back too. He's coaching again and joking at dinner. He's never had a depression or a high spell before this.

### What a skilled clinician weighs after each clue

1. Major depressive disorder, Adjustment disorder, Depressive disorder due to another medical condition, Substance/medication-induced depressive disorder
2. Major depressive disorder, Depressive disorder due to another medical condition, Substance/medication-induced depressive disorder
3. Major depressive disorder, Depressive disorder due to another medical condition
4. Depressive disorder due to another medical condition, Major depressive disorder
5. Depressive disorder due to another medical condition, Major depressive disorder
6. Depressive disorder due to another medical condition

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Depressive Disorder Due to Another Medical Condition

# Patient 24: The flight attendant



**Setting:** Leilani, 29, a flight attendant, at a telehealth visit with a social worker at her primary care clinic. Her partner, Kai, joins the video call.

> *"Kai thinks I need help. Some weeks I agree. Other weeks I think I'm fine and Kai's overreacting."*

**If asked about…**

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|---|---|
| **When did this start?** | It's been bad for about a year, every month. It slowly got worse before that. |
| **Timing** | About a week before my period. By day two or three of my period I'm me again. The week after is my good week. |
| **Symptoms that week** | Irritable, on edge, crying at anything, wiped out, craving chips, can't focus, sore breasts, bloated, overwhelmed. |
| **Sleep** | That week I want to sleep twelve hours. Other weeks it's normal. I've never gone without sleep and felt great. |
| **The daily ratings** | My doctor gave me a chart. I filled it out every night for two cycles. Same thing both times: a spike before my period, almost nothing after. |
| **Alcohol, drugs, medications** | A glass of wine on some layovers. No drugs. Off birth control pills for over two years. No medications. |
| **Medical history** | Thyroid and blood work were normal. My periods are regular. |
| **Thoughts of suicide** | That week I sometimes think 'what's the point,' but I've never wanted to die or planned anything. The other weeks I'm fine. |
| **Work** | I've called in sick twice this year during that week. On a full flight, snapping at passengers is a real problem. |
| **Relationship** | Kai and I fight that week, and I say things I regret. The rest of the month we're good. |
| **Family history** | My mom says she was 'a monster' before her period, but nobody ever treated it. |
| **Culture and context** | Growing up in Hilo, nobody in my family talked about periods, let alone moods. You just handled it. I didn't know this was something doctors treat. |

**Only if asked (don’t volunteer)**

- Symptoms come only in the week before menses and clear by day 2 or 3
- Completed two cycles of daily symptom ratings
- Off hormonal contraception for over two years

# Patient 24: answer and debrief



DIAGNOSIS

## Premenstrual dysphoric disorder

Irritability, crying and self-criticism that come and go look like a mood or personality disorder. Clue 4 flips it: the symptoms arrive the week before her period and clear within days after it starts, every month for a year. Five or more symptoms, including mood symptoms, with impairment, confirmed by two cycles of daily ratings, with no hormonal treatment and normal labs, make this premenstrual dysphoric disorder. Exam lesson: confirm PMDD with daily ratings over at least 2 cycles (provisional before that), separate it from another disorder that only worsens before menses, and ask about suicidal thoughts during the symptomatic week.

### What had to be ruled out

- Bipolar II disorder and cyclothymic disorder: no periods of elevated mood or reduced need for sleep
- Major depressive disorder: symptoms are minimal or absent in the week after menses
- Borderline personality disorder: shifts are tied to the cycle, with symptom-free weeks
- Substance/medication-induced depressive disorder: off hormonal contraception for over two years; no drugs
- Depressive disorder due to another medical condition: normal thyroid and blood work
- No mental disorder: marked distress, missed work and relationship conflict

### Checkpoint: What would you ask or check NEXT?

A. Ask whether the bad stretches follow a pattern, such as a particular time in her menstrual cycle **(best)**

B. Suggest an evaluation for a mood stabilizer to even out her mood swings

C. Tell her the pattern fits borderline personality disorder and refer her to DBT

Recurring irritable, low spells with normal weeks in between call for mapping their timing, and for someone who menstruates, the cycle is a key pattern to check. B treats a bipolar disorder that clue 3 argues against. C labels a personality disorder after one session, without a long-standing pattern.

### The case in six clues (as the app reveals it)

1. My moods are out of control. I snap at Kai, then I cry for hours, and some days I don't recognize myself.
2. Partner: Some days she's furious over nothing and says she's worthless and should quit flying. Then she's completely herself again for a couple of weeks.
3. I've never had nights where I didn't need sleep or felt on top of the world. I don't use drugs, I have a drink on layovers sometimes, and I stopped birth control pills over two years ago. I'm on no medications.
4. Partner: I started marking it on the calendar. Every month this past year, it hits about a week before her period, and by the second or third day of her period she's back to normal.
5. That week I'm irritable, on edge, crying at commercials, exhausted and craving salty food. I can't focus, my breasts ache, and I feel overwhelmed. I've called in sick twice, and Kai and I fight.
6. My doctor had me rate my symptoms every day for the last two cycles. Both times my scores spiked the week before my period and dropped to almost nothing the week after. My thyroid and blood work were normal.

### What a skilled clinician weighs after each clue

1. Borderline personality disorder, Bipolar II disorder, Major depressive disorder, Premenstrual dysphoric disorder
2. Bipolar II disorder, Cyclothymic disorder, Major depressive disorder, Premenstrual dysphoric disorder
3. Major depressive disorder, Premenstrual dysphoric disorder, Borderline personality disorder
4. Premenstrual dysphoric disorder, No mental disorder (normal or expected reaction), Major depressive disorder
5. Premenstrual dysphoric disorder, Depressive disorder due to another medical condition
6. Premenstrual dysphoric disorder

Sources: DSM-5-TR (APA, 2022), Depressive Disorders: Premenstrual Dysphoric Disorder